

HIPROGIN 250MG INJECTION 1ML

DESCRIPTION: Hiprogin 250mg injection 1ml: A clear yellow oily solution for injection.

COMPOSITION: Each ml contains Hydroxyprogesterone Caproate 250 mg.

PHARMACODYNAMICS: Progestins enter target cells by passive diffusion and bind to cytosolic (soluble) receptors that are loosely bound in the nucleus. The steroid receptor complex initiates transcription, resulting in an increase in protein synthesis.

Progestins are capable of affecting serum concentrations of other hormones, particularly estrogen. Estrogenic effects are modified by the progestins, either by reducing the availability or stability of the hormone receptor complex by turning off specific hormone-responsive genes by direct interaction with the progestin receptor in the nucleus. In addition, estrogen priming is necessary to increase progestin effects by upregulating the number of progestin receptors and/or increasing progesterone production, causing a negative feedback mechanism that inhibits estrogen receptors.

Depending on the progestin and its dose, progestin may demonstrate varying degrees of progestational effects. Also, other hormonal effects, such as estrogenic, anabolic, androgenic, or glucocorticoid-inducing or suppressing effects are demonstrated to different degrees and depend on the progestin type and dose.

Progestational agents - Progestins produce histologic changes in the vaginal epithelium and induce secretory-like changes in the endometrium in the second half of the menstrual cycle. As progestin levels fall after estrogen priming, uterine bleeding occurs.

PHARMACOKINETICS: Hydroxyprogesterone caproate is found in the blood only 2 hours after IM administration. Maximum plasma levels are reached on the 2nd day of injection. After that, the concentration decreases slowly 14 days after injection, 60% of the maximum value can still be found. The elimination is protracted. Hydroxyprogesterone caproate is eliminated entirely in the form of conjugated metabolites. About 80% via the liver, about 20% via the kidney. Hydroxyprogesterone caproate is more potent than progesterone with longer duration of action. Synthetic 17-hydroxy derivative of progesterone with progestogenic, androgenic, and glucocorticoid effects.

INDICATION:

1. Habitual abortion, when associated with proven progesterone-deficiency.
2. Primary and secondary amenorrhoea.
3. Infertility due to corpus luteum insufficiency, primary and secondary amenorrhoea.

RECOMMENDED DOSAGE:

Habitual abortion, when associated with proven progesterone-deficiency: 250-500 mg by intramuscular injection at weekly intervals during the first half of pregnancy.

Threatened abortion: The therapy is initiated with an i.m. injection of 500 mg Hiprogin Injection 2 - 3 times weekly until the bleeding ceases, bed rest being urgently recommended. The treatment must then be continued for several weeks with 250 mg Hiprogin Injection i.m. twice weekly until the patient remains free from complaints and bleeding despite mobilization. Whether Hiprogin Injection should be given prophylactically even beyond this point will depend on the individual case.

8 to 14 days after unsuccessful treatment of threatened abortion and subsequent curettage withdrawal bleeding may occur in isolated cases owing to the continuing effect of Hiprogin Injection which subsides only gradually. However, no further measures are necessary.

Infertility due to corpus luteum insufficiency: In cases in which the luteal phase is short - a situation characterized by too short-lived an increase in the basal body temperature in the second half of the cycle - Hiprogin Injection brings about secretory transformation of the inadequately transformed endometrium, thus improving the chances of nidation.

Intramuscular injection of 250 mg Hiprogin Injection about 3 days after the rise in basal body temperature. Since there is often a coexisting oestrogen deficit, additional administration of an oestrogen-preparation. Thus a physiological transformation of the endometrium can be achieved.

Primary and secondary amenorrhoea: In the case of secondary amenorrhoea hormonal treatment is to be given at the earliest 8 weeks after the last menstrual period. In order to induce a menstruation-like bleeding, oestrogen-preparation is to be given before the administration of Hiprogin Injection. However, before treatment is commenced the presence of a prolactin-producing pituitary tumour should be excluded because, according to the present state of knowledge, the possibility cannot be ruled out that macroadenomas increase in size when exposed to higher doses of oestrogen for prolonged periods of time.

Commencement of treatment: On the first day of treatment an oestrogen-preparation, 14 days later an oestrogen-preparation together with 250 mg Hiprogin Injection intramuscularly. Withdrawal bleeding starts about the 28th day of treatment.

Continuation of treatment (over at least 2 - 3 cycles): On the 6th day of the artificial cycle, an oestrogen-preparation and on the 16th day an oestrogen preparation together with 250 mg Hiprogin Injection intramuscularly (1st day of bleeding = 1st day of cycle).

An attempt can then be made to stop the oestrogen-treatment, and to induce a cyclical bleeding by an intramuscular injection of 250 mg Hiprogin Injection between the 18th and the 20th day of the cycle.

Exception: Patients of whom it can be safely assumed that endogenous oestrogen production is insufficient (primary amenorrhoea in gonadal dysgenesis).

Please note: If there is no desire for children, contraception should be practised with non-hormonal methods (with the exception of the rhythm and temperature methods). If withdrawal bleeding at regular intervals of about 28 days fails to occur under the therapeutic scheme pregnancy must be considered despite the protective measures. The treatment must then be interrupted until the situation has been clarified by differential diagnosis.

If, on the other hand, there is a desire for children and pregnancy has occurred, treatment with Hiprogin Injection should only be continued if there is assume a risk of abortion.

ROUTE OF ADMINISTRATION: Intramuscular administration

CONTRAINDICATIONS: Known or suspected breast malignancies or tumours. Acute hepatic disease including benign or malignant liver tumours. Known or suspected pregnancy. Sensitivity to progestins. Undiagnosed urinary tract bleeding. Undiagnosed uterine or genital bleeding. A history of herpes gestationis.

WARNINGS & PRECAUTIONS:

Patient with diabetes mellitus - High dose of progestins may alter carbohydrate metabolism by an unknown mechanism, producing a mild decrease in glucose tolerance in some patients; progestin-only contraception does not usually affect carbohydrate metabolism. Time to start and maximal effects may differ between progestins and their doses with glucose are seen in nondiabetics receiving low doses.

As this product contains Benzyl Alcohol, its use should be avoided in children under two years of age. Not to be used in neonates.

INTERACTION WITH OTHER MEDICAMENTS:

Hepatic enzyme inducing medications, such as:

Carbamazepine or phenobarbital or phenytoin or rifabutin or rifampin (decreased efficacy of some progestins, including levonorgestrel implants, has been suggested to be caused by enhanced metabolism of the progestins by these drugs).

(Phenytoin and rifampin increase the serum concentrations of sex-hormone binding globulin (SHBG); this significantly decreases the serum concentration of free drug for some progestins).

(Drug interaction data are not available for rifabutin, but because its structure is similar to that of rifampin, similar precautions with its use with progestins may be warranted).

The requirement for oral antidiabetics or insulin can change.

USE IN PREGNANCY & LACTATION:

Fertility - Progestins cause a decrease in quantity and/or change the quantity of cervical mucus and may interfere with sperm function, fertilization, and subsequently, the occurrence of pregnancy. This effect depends on the dose and type of the progestin. High-dose or long-term use of progestins may cause a delayed return to fertility.

Use in Pregnancy: Progestins, in general, should be withheld during pregnancy. Progestin cross the placenta. Although many studies fail to demonstrate an increase in teratogenicity when progestins are given in the first trimester, the possibility that genital abnormalities may appear in male and female fetuses exposed to progestins during that period has been suggested.

Ectopic pregnancy is possible with contraception failure because some progestin-only contraceptives protect mainly against intra-uterine pregnancy. Hiprogin should be used to maintain pregnancy only if it is strictly indicated, ie. if there is an urgent desire to have a child, and luteal insufficiency is present.

Before starting the use of Hiprogin Injection a thorough general medical and gynaecological examination (including the breasts and a cytological smear of the cervix) should be carried out and pregnancy must be excluded in the appropriate indications.

Use in Lactation: Progestins are distributed into breast milk in variable amounts and, depending on the progestin and dose, may increase or decrease quantity or quality or have no effect on breast milk.

SIDE EFFECTS: For all progestins, if abnormal bleeding is persistent (longer than 10 days at a time) or recurring (heavier than normal menses occurring longer than 10 months after beginning therapy or more than monthly) malignancy should be considered as a cause of the bleeding. Rarely, local reaction may occur at the site of injection, coughing, dyspnea.

SYMPTOMS AND TREATMENT OF OVERDOSE:

No data on overdosage available, but if adverse effects such as thromboembolic disorders or ocular disorders occur, the medication should not be readministered.

Side effect of Hydroxyprogesterone Caproate – see under Medroxyprogesterone Acetate.

INCOMPATIBILITIES: Unknown

STORAGE CONDITIONS: Store below 30°C. Protect from freezing. KEEP OUT OF REACH OF CHILDREN. *JAUHKAN DARIPADA KANAK-KANAK.*

SHELF LIFE: Please refer to outer package.

PACK SIZE: Pack in box of 10 x 1ml ampoule

PRODUCT REGISTRATION HOLDER & MANUFACTURER:

DUOPHARMA (M) SDN BHD

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